Structure and Contents of the Medical Record

Which documents make a complete file, in what order, and why uncontrolled forms are a safety problem.

Module	02 of 12
Study time	95 minutes
Audience	Medical records staff, nursing, clinical staff, quality teams, forms committee members
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	NABH 6th Edition IMS chapter; AAC chapter; hospital forms control practice

A complete medical record is not an arbitrary bundle of whatever the ward generated. It is a defined set of documents, assembled in a fixed order, each of them a controlled form carrying the patient's identifiers. This module covers what must be in an inpatient file, the order it goes in, what must appear on every page and every entry, and why a photocopied form with no version number is a genuine clinical risk rather than a filing irritation.

LEARNING OUTCOMES — by the end of this module you will be able to:

- List the documents that make up a complete inpatient medical record
- Assemble a file in the standard order and explain why a fixed order matters
- State what must appear on every page and what must appear against every entry
- Distinguish the outpatient, emergency, day-care and inpatient record sets
- Explain forms control: design, approval, numbering, version control and withdrawal
- Identify an uncontrolled form and describe the risk it creates
- Apply a deficiency checklist to an assembled file

2.1 Why structure matters more than it appears to

In an emergency, nobody reads a file; they find a page. A clinician called to a deteriorating patient needs the drug chart, the last set of vitals and the allergy status within seconds. If every ward assembles files differently, that search takes minutes, and in the meantime the clinician acts on memory and on what the nearest person tells them. Standard structure is a patient-safety control, not a clerical preference.

Structure also determines whether a file can be audited at all. A reviewer with a checklist needs to know where each document should be, so that absence is detectable. In an unstructured file, a missing consent form looks exactly like a consent form that is somewhere else.

2.2 The standard assembly order

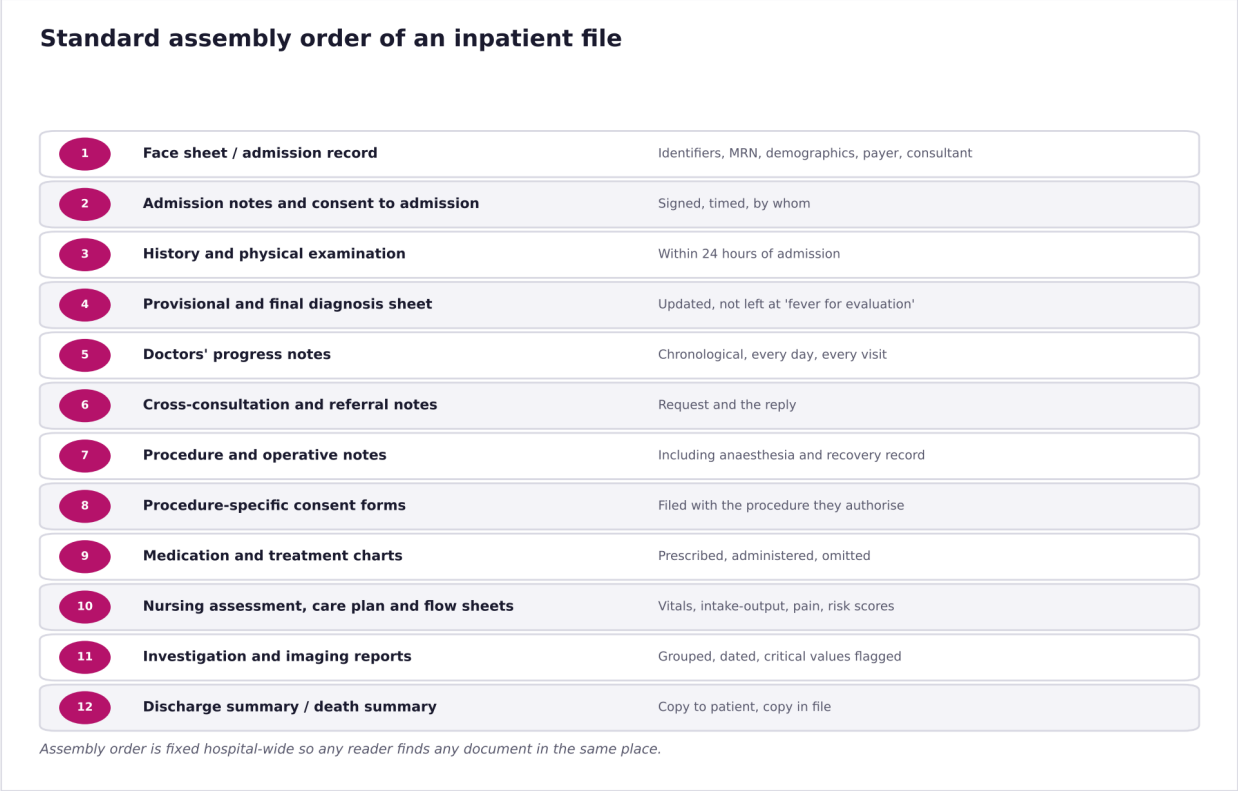


Figure 2.1 — Standard assembly order for an inpatient file.

Two orders are in use in practice and hospitals must choose one and apply it everywhere. **Chronological order** — oldest at the back, newest at the front — suits the active file on the ward, because the newest entry is the one being written. **Assembly order** — the fixed sequence shown above — suits the discharged file, because a reader looking for a specific document finds it in the same place in every file. Most hospitals run the ward file in reverse chronological order and reassemble it into standard order in the medical records department after discharge. What is not acceptable is each ward deciding for itself.

> **KEY POINT**
> Write the assembly order into policy, print it, and fix it inside the front cover of the file jacket. An order that exists only in the head of the senior MRD clerk disappears the day they take leave.

2.3 The mandatory contents of an inpatient record

Document	What it must contain at minimum
Face sheet / admission record	Full name, medical record number, age, sex, address and contact, admission and planned discharge details, consultant and department, payer or scheme, emergency contact, and the admission diagnosis
Consent to admission and to treatment	General consent to admission and routine treatment, signed and dated by the patient or authorised person, with the person obtaining it identified
Initial medical assessment	Presenting complaint, history, past and family history, allergies, medication history, examination findings, provisional diagnosis, differential diagnoses considered, and the initial plan
Initial nursing assessment	Nursing history, functional and nutritional status, risk screens for falls, pressure injury, pain and where applicable VTE, with the scores recorded

Document	What it must contain at minimum
Diagnosis sheet	Provisional diagnosis at admission and final diagnosis at discharge, updated rather than left unchanged, with co-morbidities and complications listed separately
Doctors' progress notes	Dated and timed entries for every clinical contact, recording findings, assessment and plan, signed with name and registration number
Nursing notes and flow sheets	Shift notes, vital signs, intake and output, pain scores and reassessment, risk-score reviews, and the care plan with its revisions
Medication and treatment charts	Allergy status, prescriptions with all required elements, administration signatures, omission codes, and records of high-alert drug checks
Investigation and imaging reports	All reports relating to the admission, grouped and in date order, with critical values identifiable
Procedure, operative and anaesthesia records	Pre-anaesthetic assessment, procedure-specific consent, operative note, anaesthesia record, implant details, and the post-anaesthesia recovery record
Cross-consultation and referral notes	Both the request, stating the clinical question, and the reply, stating the opinion and the recommendation
Discharge summary or death summary	The full contents set out in Module 8, authenticated, with evidence that a copy was given to the patient or the family

Some documents are conditional rather than universal: restraint records, resuscitation records, transfusion records, DAMA forms, incident reports relating to the patient, medico-legal documents, and research consent. The file index should list these so that a reviewer can see whether the right conditional documents are present for the care actually delivered.

2.4 What goes on every page and against every entry

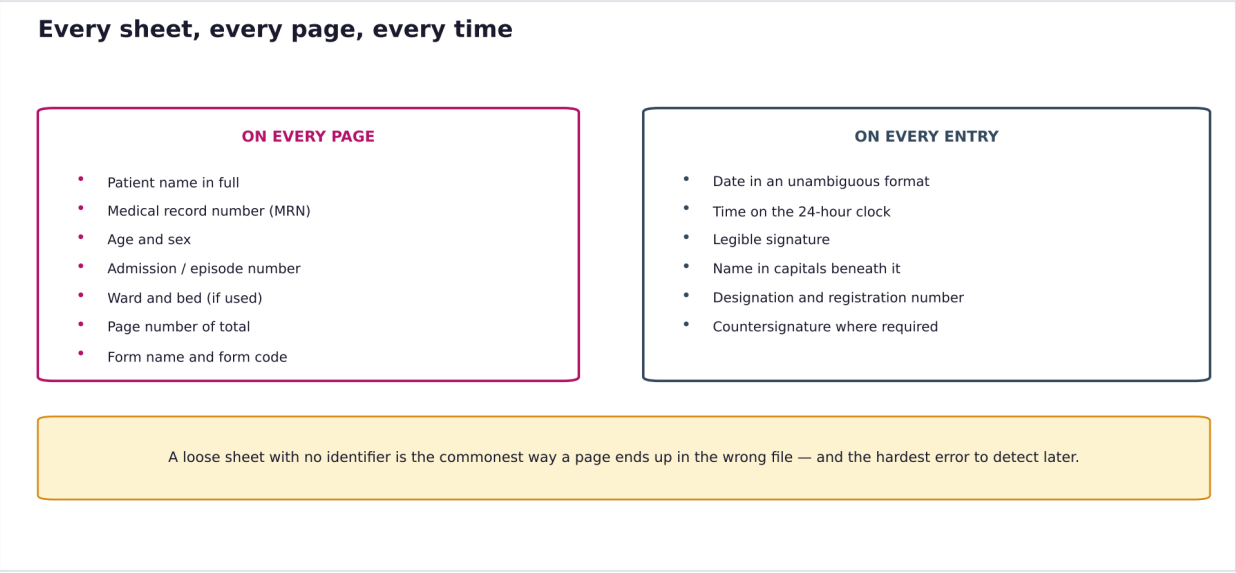


Figure 2.2 — Page identification and entry authentication requirements.

Page identification exists to solve one problem: loose sheets. Pages come out of files for photocopying, for the ward round, for the coder, for the claim. A sheet with no identifier that rejoins the wrong file is almost undetectable later, and the consequences range from a confusing audit to a drug given on the basis of another patient's result.

Entry authentication exists to solve a different problem: accountability. An entry nobody can be identified with cannot be relied upon, cannot be questioned, and in professional proceedings it tends to be attributed to the most senior person who might have written it. The full set is signature, name in capitals, designation, registration number

where applicable, date and time.

> **CAUTION**
> Time is the element most often omitted and most often decisive. Disputes about clinical care are overwhelmingly disputes about sequence: what was known when, how long the deterioration went unnoticed, how quickly the escalation happened. An undated and untimed entry cannot answer any of those questions.

2.5 Outpatient, emergency and day-care records

Hospitals concentrate documentation effort on inpatient files and then lose cases on outpatient and casualty records, which are typically thinner, more rushed, and far more numerous.

Setting	Minimum record set	Where it usually fails
Outpatient	Date, identifiers, presenting complaint, relevant history and examination, diagnosis or working impression, investigations ordered, treatment advised, follow-up, prescriber identification	No examination findings; diagnosis absent or recorded only as a symptom; no record of what the patient was advised; prescriptions issued without the record showing why
Emergency	Time of arrival, triage category and time, initial assessment, vitals, treatment with times, reassessment, disposal decision with reason, time of departure, condition and advice at discharge, referral or admission details	Triage time not recorded; no reassessment before discharge; return advice not documented; medico-legal screening missed
Day care and day surgery	Pre-procedure assessment and fitness, consent, procedure record, sedation or anaesthesia record, recovery observations, discharge criteria met, escort arrangements, post-procedure instructions and contact number	No documented discharge criteria; instructions given verbally only; no record of who escorted the patient home
Ambulatory chemotherapy or dialysis	Protocol and cycle or session number, pre-treatment parameters, consent, drug or dialysis prescription, administration record, observations during and after, adverse events, next session plan	Cumulative dose not tracked; adverse events recorded in the nursing note but never in the clinical record

2.6 Forms control

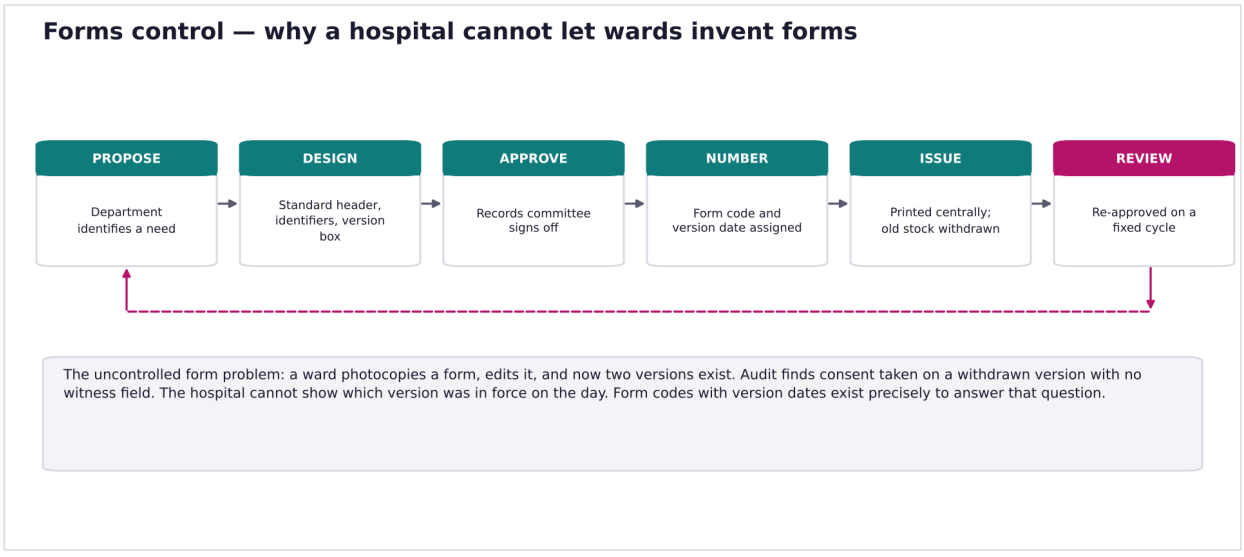


Figure 2.3 — The forms control cycle.

A hospital form is a controlled document. That means it has an owner, a code, a version, an approval date and a review date, and that only the current version is in circulation. The alternative — wards photocopying and editing forms as needs arise — produces a situation where the hospital cannot say which version of a consent form was in force on the day a disputed consent was taken.

- Every form carries a code and a version date, printed in a consistent position, usually bottom left or bottom right
- Every form carries the standard patient identification block in the same position
- A master list of forms is maintained, showing code, title, version, approval date, owning department and review date
- Forms are printed or issued centrally; wards cannot originate stationery
- When a version is superseded, old stock is withdrawn and destroyed, and the withdrawal is recorded
- Obsolete versions are retained in the master file only, so that a historical form can be identified later
- Forms are reviewed on a fixed cycle, and changes go through the committee rather than through whoever has access to the file

How to spot an uncontrolled form

- No form code, or a code that does not appear on the master list
- No version date, or a version date earlier than the current approved version
- A photocopy of a photocopy, with degraded edges and a handwritten amendment
- Fields crossed out or added in pen on the printed form
- A form in use in one ward and unknown in the next
- A form carrying the logo of a drug company or a device supplier

> **CAUTION**
> A consent form that has been locally amended is the highest-risk uncontrolled document in a hospital. If the amendment removed a field — the witness line, the risks section, the name of the person explaining — the hospital has no answer when the omission is pointed out in proceedings.

2.7 The file jacket and the index

The jacket is more than a cover. It carries the medical record number in a position readable from the shelf, the patient’s name, and in terminal-digit filing the colour-coded digits that make misfiles visible. It should also carry a contents index and a record of each episode — admission dates, discharge dates and the consultant — so that a clinician opening a thick unit file can locate the relevant episode without reading the whole file.

Flags on the jacket matter: medico-legal, research participation, special categories such as HIV or mental health where additional confidentiality rules apply, and known allergies. These flags must be applied by rule rather than by judgment, and must be visible without opening the file.

2.8 The deficiency checklist

Analysis in the medical records department is only as good as the checklist it uses. A checklist that says “record complete: yes / no” generates a number and no improvement. A usable checklist is itemised, objective and tied to a named owner for each deficiency.

Checklist item	Deficiency recorded against
Face sheet complete, including final diagnosis	Treating unit
Initial medical assessment present and within twenty-four hours	Admitting doctor
Initial nursing assessment present and within the policy time	Ward nursing in-charge

Checklist item	Deficiency recorded against
Progress note present for every day of stay	Treating unit
All entries signed, dated and timed	The individual author, aggregated by unit
Consent present for every procedure performed, correctly completed	Proceduralist
Operative note present and complete	Operating surgeon
Anaesthesia and recovery records present and complete	Anaesthetist
Medication chart complete; allergy status recorded; verbal orders countersigned	Prescriber
Investigation reports filed and critical values acted upon in the notes	Treating unit
Discharge or death summary present, complete and authenticated	Treating unit
No prohibited abbreviations used	The individual author, aggregated by unit
No obliterated or improperly corrected entries	The individual author, aggregated by unit
All pages carry patient identifiers; no loose or unidentified sheets	Ward nursing in-charge and MRD

2.9 Paper, electronic and the hybrid trap

Most Indian hospitals run hybrid records: some documents electronic, some on paper, and the boundary varying by department and by time of day. Hybrid is not a transitional inconvenience; it is the highest-risk configuration, because no single reader can see the whole record.

Risk	How it appears	Control
Nobody holds the complete record	A clinician reads the EMR and misses the paper consent; an auditor reads the file and misses the electronic order	A documented map stating exactly which document lives where, available to every clinician
Duplicate and conflicting entries	Allergies recorded in both systems and disagreeing	A single authoritative source for each data element, stated in policy
Scanned documents arriving late	Paper scanned days after discharge, so the record is incomplete when the claim or the query is raised	A scanning turnaround standard, measured
Printing and re-filing	Printed EMR pages filed loose into the paper record, creating uncontrolled copies	A rule on what may be printed, with printed copies marked as such and a retention rule for them
Version confusion	A printed medication chart continues in use after the electronic one is updated	Only one chart in force at a time, with the changeover documented

> **CAUTION**
> In a hybrid hospital, the question ‘where is the complete record for this admission?’ must have a written answer. If three people give three different answers, the hospital cannot produce a complete record on demand, which is a failure before any clinical question arises.

2.10 Designing a form that gets completed

Form design is a documentation control. A form that is hard to complete correctly will be completed incorrectly, and no amount of training fixes a bad form.

- Put the patient identification block in the same position on every form, at the top
- Order the fields in the sequence the clinician actually works, not in the sequence the committee thought of them

- Give each field enough physical space for the answer; a two-centimetre box invites an abbreviation
- Avoid fields that can be left blank ambiguously; use explicit ‘yes / no / not assessed’ rather than a tick box that means nothing when empty
- Put the signature block at the end of each section, not only at the end of the form, so that a partially completed form is still attributable
- Print the form code and version date in a consistent position
- Pilot a new form in one ward for a month and audit the completed copies before rolling it out
- Count the forms a nurse must complete for one admission; if the number is above what a shift allows, the forms are the problem and not the nurse

2.11 Specialty record sets

Several specialties require documents beyond the general inpatient set, and these are the documents most often absent because they sit outside the standard checklist.

Specialty	Additional documents the record must contain
Obstetrics	Antenatal record, partogram, fetal heart monitoring traces with times, delivery record, placental examination, newborn record with APGAR scores, and the maternal observation chart for the postnatal period
Paediatrics and neonatology	Growth charts, immunisation record, weight-based dose calculations shown, feeding charts, and the parent or guardian identified for every consent
Psychiatry	Capacity assessment, advance directive where it exists, nominated representative, restraint and seclusion records with review intervals, and the statutory admission category
Oncology	Protocol and cycle number, body surface area and dose calculation, cumulative anthracycline or other dose tracking, pre-chemotherapy parameters checked, and extravasation records
Dialysis	Session record with pre- and post-weights, access assessment and complications, dialysis prescription with the parameters actually delivered, and periodic adequacy testing
Intensive care	Hourly flow sheets, ventilator and infusion settings with the reason for each change, device insertion and removal dates, daily goals, and family communication records
Emergency	Triage record with time and category, reassessment, medico-legal screening, and disposal decision with the reason
Transplant	Statutory authorisation forms, donor and recipient documentation kept separately, committee approvals, and tissue traceability

2.12 Loose sheets and how documents are lost

Documents do not usually vanish; they detach. Understanding the detachment points is how a hospital stops losing them.

Detachment point	Why it happens	Control
Photocopying for a claim	Pages removed and returned in the wrong order or not at all	Copying done in the records department, never by removing pages from a file in a ward
The ward round	Pages pulled out for discussion and left on a trolley	A rule that the file travels intact; nothing is removed from the jacket on the ward
Reports arriving after discharge	The file has gone to the department and the report has nowhere to go	A defined late-document route, with the report matched by record number and filed with an entry noting when it arrived

Detachment point	Why it happens	Control
Theatre and procedure areas	Records generated away from the ward and sent separately	Documents accompany the patient back; the receiving nurse checks against a list
Billing and insurance	Documents sent out and not returned	Only certified copies leave; the original file never goes to billing
Investigations department	Requisitions and reports moving independently of the file	Electronic linkage by record number; printed reports delivered to the records department, not to individuals

2.13 Registration: where the record begins

Every defect in a medical record that relates to identity originates at registration. A name entered differently from last time creates a duplicate; an age estimated rather than asked corrupts every age-based calculation; a missing contact number means the hospital cannot reach the patient when a result arrives.

Field	Why it matters later	How it is usually got wrong
Full name	The primary search key, and the identifier on every page	Initials expanded differently between visits; surname and given name transposed; transliteration varying
Age and date of birth	Paediatric dosing, age-based protocols, retention of minors' records, and statistical reporting	Age estimated by the clerk; date of birth defaulted to 1 January
Sex	Clinical protocols, reference ranges, and ward allocation	Assumed from the name rather than asked
Father's or spouse's name	A key discriminator between patients with common names	Left blank because it is treated as optional
Address and contact number	Reaching the patient about a result, a recall or a follow-up	A number that belongs to an accompanying person who is not traceable later
Government identifier or ABHA number where captured	Linking records across visits and across institutions	Captured inconsistently, or captured without being verified
Payer and scheme details	The claim and the pre-authorisation route	Captured at discharge rather than at admission, by which time pre-authorisation has been missed

> **KEY POINT**
> Registration staff are the first link in the documentation chain and are usually the least trained. An hour spent on search discipline and name standardisation at registration prevents more record problems than any downstream control.

2.14 The ward check before a file leaves

A short, structured check on the ward, performed before the file goes to the medical records department, closes most deficiencies while the people who can close them are still present. It takes a few minutes and it removes the great majority of the chase that follows.

1. Every page carries the patient's name and record number; no loose or unidentified sheets.
2. The initial medical and nursing assessments are present.
3. There is a progress note for every day of the admission.
4. Every consent form required by the procedures performed is present and complete.

5. The operative, anaesthesia and recovery records are present for every procedure.
6. The medication chart has the allergy field completed and no unsigned administration boxes.
7. All investigation reports belonging to the admission are filed.
8. The discharge or death summary is present, complete and authenticated, and the patient's copy is acknowledged.
9. Any outstanding signature is obtained before the file leaves the ward, not after.

Hospitals that make this check a nursing handover responsibility, rather than an additional task, see completion rates rise without any additional staffing, because the check happens while the clinicians are still on the ward.

2.15 Putting this module to work

1. Pull three discharged files from your ward and check whether they are assembled in the same order. If they are not, there is no assembly standard in practice, whatever the policy says.
2. Take ten consecutive pages from one file and check every page for the patient's name and record number.
3. Look at the bottom of five different forms in use on your ward. Do they carry a code and a version date, and are those versions on the master list?
4. Find your hospital's deficiency checklist. If each item does not name an owner, propose that it should.
5. Check whether the file jacket carries medico-legal and allergy flags, and whether the flag is applied by rule or by whoever happens to notice.

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A ward keeps files in reverse chronological order and the MRD reassembles them into standard order after discharge. A clinician complains that this is inconsistent. Is the hospital wrong?**

Answer: No, provided both orders are defined in policy and applied uniformly. The active file is read forward from the newest entry, so reverse chronological order serves the ward; the discharged file is searched for specific documents, so assembly order serves every later reader. What matters is that neither order is left to individual preference and that the point of changeover is defined and documented.

Q2. **You find a consent form in use in one theatre that has an extra handwritten line and no version date. What is the risk and what should be done?**

Answer: It is an uncontrolled document. The hospital cannot establish which version was in force on any given date, and if a field has been removed or altered, the consent itself becomes contestable. Withdraw the form from use immediately, retrieve the stock, identify how it originated, check whether any consents have already been taken on it, refer the proposed change to the forms committee, and reissue only the approved version with a code and version date.

Q3. **An auditor finds a laboratory report in a file bearing a different patient's name. How does this happen and what controls prevent it?**

Answer: It happens when reports are filed in bulk without checking identifiers, or when a loose sheet without identifiers is filed into the nearest open file. The controls are: every page carrying full patient identifiers printed at source; reports matched to the file by record number and not by name alone; a check at filing; and an analysis step in the MRD that would detect a mismatched document. The incident should also be reported, because the risk is that a clinical decision was taken on another patient's result.

Q4. **Your emergency department records show time of arrival and time of departure but no triage time and no reassessment before discharge. Which standards does this breach and what is the clinical risk?**

Answer: It breaches the AAC requirements on initial assessment and reassessment and weakens the whole emergency record. Clinically, the absence of a triage time means the hospital cannot show how long a patient waited with a time-critical presentation, and the absence of reassessment means it cannot show that the patient was safe to discharge. Both are the exact points on which emergency cases are lost.

Q5. **A day-surgery patient returns with a complication and alleges they were discharged too early and given no instructions. The file shows a procedure note and a discharge slip with no observations and no instructions. What was missing?**

Answer: The recovery observations, the documented discharge criteria with the time they were met, the written post-procedure instructions with a contact number, the record of what was explained and to whom, and the escort arrangements. Day-care documentation is often treated as lighter than inpatient documentation; in disputes it is held to the same standard, and the discharge decision is the part that is scrutinised.

Q6. **The MRD reports that eighty per cent of files are complete. The quality team asks what should be done. What is wrong with the question?**

Answer: The figure is not actionable on its own. A usable report says which items were deficient, in which units, and against which owners — for example, that the twenty per cent is driven almost entirely by missing operative notes in two theatres and unsigned nursing entries on night shifts. Completeness as a single percentage tells you only that there is a problem, not what to change.

ASSESSMENT BANK
A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.